

China should improve the diagnostic and treatment mechanism for ADHD in minors

According to the epidemiological survey report of China's National Health Commission in 2024, the prevalence of ADHD in children aged under 18 is 6.4%. At the same age group, according to ABC, ADHD affects approximately 281,000 children and adolescents, representing around 8–10% of the youth population in Australia. Why are the data in China lower than those in Australia? The underlying reason is that China's diagnostic mechanism for ADHD in minors is extremely inadequate. In other words, this is not because China has done a good job in preventing ADHD in children. On the contrary, it is because there is a lack of a systematic and standardized diagnosis and treatment system.

In terms of medical resources and treatment, the medical and information resources available for treating ADHD in minors in China are insufficient, and the distribution of these resources is extremely uneven. According to Frontiers, there are approximately 3,000 registered pediatric and adolescent psychiatrists in China, but the total number of children diagnosed with ADHD is 23 million, in other words, the ratio of doctors to patients is 1:7,700. The Royal Australian and New Zealand College of Psychiatrists states that there are approximately 450 registered child and adolescent psychiatrists in Australia, who are required to serve around 560,000 children with ADHD, resulting in a ratio of about 1:1,310. Although the expert shortages in both countries are significant, the shortage in China is 5.8 times that of Australia. Meanwhile, Qian et al. argue in Springer Nature that 41.4% of the regions have no mental health institutions at all in China, and these areas are mostly rural and economically underdeveloped regions. Furthermore, in China, there are only some investigation reports regarding the diagnosis and treatment of ADHD in minors, and there is no complete database. This aspect is also seriously lacking, too.

In terms of the willingness to diagnose and intervene for adolescent ADHD, the situation in China is even more problematic.

Survey from PubMed Central (PMC) found that in China, only about 10% of ADHD patients seek medical treatment at hospitals. This means that 90% of the patients do not enter the medical system. Therefore, the number of diagnosed cases is actually much lower than the actual number of patients. PMC claim that is because in China, mental illness is often heavily stigmatized and associated with social shame and loss of face, which makes parents unwilling to acknowledge their children's psychological problems or seek help. And academic achievement is often prioritized over psychological well-being, leading parents to overlook mental health problems in children. Various factors have led to a further decrease in the treatment coverage rate: according to the data from PMC and ADHD Evidence Project, among children who seek medical care, only about one third receive medication treatment. Overall, the situation is even more limited. In China, only about 0.036% of children aged 0–17 are treated with medication for ADHD. This means that, on average, only around one out of every 200

children with ADHD is receiving medication. According to data from the Australian Institute of Health and Welfare among children diagnosed with ADHD in Australia, about 10% to 54% (See Appendix A) are receiving medication treatment. Although this range is quite broad, it still far exceeds the data of China.

Some may argue that China's lower diagnosis rate reflects a genuinely lower prevalence of ADHD, suggesting that fewer medical resources are therefore required. Others believe that expanding the diagnostic system may lead to over-diagnosis, unnecessary medication treatment, and greater pressure on families. However, these arguments do not take into account the relevant evidence of a large number of undiagnosed and unmet medical needs. With only around 10% of affected children entering the healthcare system and an extremely low treatment rate of 0.036%, the issue is clearly not over-diagnosis but insufficient access and awareness. In addition, compared to Australia, there is a significant gap in the ratio of doctors to patients here, which further indicates a systemic problem rather than a decrease in the incidence rate.

China's current approach to ADHD in minors reveals a critical gap between need and support, especially compare the situation with Australia. Low diagnosis rate and low treatment rate do not mean that the incidence rate of the disease is low, but that there are systematic defects in access to medical resources, professional ability and public awareness. To address this issue, China must improve its diagnostic system, expand professional resources, and reduce discrimination to ensure that vulnerable children receive timely and effective assistance.

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Appendix

Appendix A:

Explanation about the data: in Australia, about *10% to 54%* are receiving medication treatment.

There is no relevant paper that directly informs me of this figure, which is “10% to 54%”, however, I have found the range of the number of children in Australia suffering from ADHD, as well as the approximate number of children who are receiving medication treatment. By dividing the two numbers, we obtained a broad range of the data, although the accuracy was not very high, but it can also clearly show that the ratio is many times higher than the data in China.